

Chronic Cutaneous Graft-Versus-Host Reaction After Allogeneic Transplantation

The lichenoid form of graft-versus-host disease (GVHD) closely resembles lichen planus, characterized by erythematous to violaceous, polygonal, and variably keratotic papules that typically arise on flexor surfaces. Over time, some patients develop more widespread involvement. The lichenoid phase often resolves with post-inflammatory hyperpigmentation.

The sclerodermoid phase closely mimics scleroderma, presenting with plaques of thickened skin that have a shiny, somewhat atrophic appearance, along with variable hyperpigmentation and hypopigmentation. These plaques show erythema and progressive loss of the pilosebaceous apparatus. Clinical severity varies: some patients develop only a few sclerodermoid plaques and remain asymptomatic, while others progress to widespread disease associated with joint contractures, chronic cutaneous ischemia, erosions, alopecia, and a syndrome of general wasting. Recurrent bacterial skin infections are common, reflecting the underlying immunodeficiency that characterizes chronic GVHD. This immunocompromise predisposes patients to sepsis, a frequent cause of death.

The distinction between the lichenoid and sclerodermoid phases of chronic GVHD is not absolute. There is significant overlap between the two forms in many patients. Moreover, lichen planus-like eruptions can occur early after transplantation. Given the influence of immunomodulation and variability in histocompatibility on clinical presentation, rigid classification of GVHD phases based solely on time may not be clinically appropriate.

Histopathology of Chronic Cutaneous Graft-Versus-Host Reaction after Allogeneic Transplantation

Histopathologic examination of lichenoid chronic GVHR may reveal fully developed features indistinguishable from lichen planus.